

REGULATORY COMMENT INTELLIGENCE

Public Comment Analysis

CY 2027 Medicare Physician Fee Schedule Proposed Rule

Docket CMS-2026-2377 | Document CMS-2026-2377-0002

1,595 posted comments	456 full-text sample	28.6% sample coverage
Aug. 3, 2026 snapshot date	Sept. 14, 2026 comment deadline	Live docket not a final corpus

Snapshot retrieved August 3, 2026 at approximately 8:00 PM EDT

Executive summary

This report analyzes the public comments posted to CMS-2026-2377-0002 as of the snapshot time. The live census contained 1,595 comment records. Because Regulations.gov exposes names, organizations, comment text, and attachments only through detail records—and the official public API imposed a quota during collection—the substantive analysis uses a reproducible, date-spanning detail sample of 456 records (28.6% of the census). All estimates and limitations are labeled accordingly.

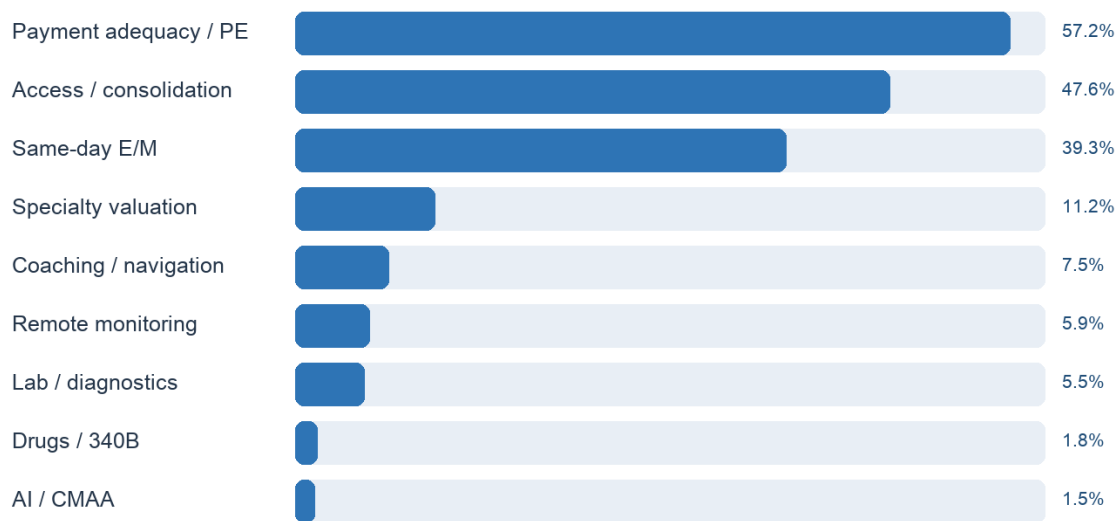
Three ideas dominate. First, commenters argue that conversion-factor reductions and practice-expense changes are incompatible with rising labor, technology, insurance, supply, and compliance costs. Second, clinicians—especially in office-based procedural specialties—oppose the proposed 50% payment for additional same-day E/M or procedure services. Third, commenters link both policies to patient-access consequences: split visits, extra travel and copays, longer waits, faster consolidation, and migration to higher-cost hospital settings.

Smaller but coherent clusters address remote monitoring staffing and initiating-visit requirements; specialty-specific code valuation; chronic disease coaching and navigation; diagnostic and laboratory policy; autonomous clinical AI payment; telehealth and safety-net clinics; wound care and skin substitutes; pharmacy and 340B issues; and Medicare eligibility. Many submissions are campaign or template letters, so raw volume should not be read as the number of independent policy arguments.

Headline findings

- Payment adequacy and practice expense appeared in 261 of 456 sampled comments (57.2%).
- Access, rural burden, or consolidation appeared in 217 comments (47.6%).
- Same-day E/M, modifier 25, or global-procedure policy appeared in 179 comments (39.3%).
- The most common requested action was to withdraw, narrow, or phase in payment reductions rather than finalize them as proposed.
- Template campaigns were visible around independent-practice payment, physical therapy, dermatology same-day care, joint arthroplasty, nurse coaching, and autonomous AI billing.

Most prevalent themes in the 456-comment detail sample



Multi-label coding; percentages sum to more than 100%. Attachment-only text not extracted.

Figure 1. Theme prevalence in the detailed sample. Multi-label coding; categories overlap.

Scope and rule context

CMS-1848-P proposes CY 2027 Medicare Physician Fee Schedule and related Part B policies. CMS states that the proposed non-QP conversion factor is \$32.84 (-1.68%) and the QP conversion factor is \$33.17 (-1.19%) versus CY 2026. The proposal would pay the most expensive same-day service at 100% and other qualifying same-day surgical or E/M services at 50%. It also proposes remote-monitoring conditions, practice-expense methodology changes, chronic-disease and behavioral-health policies, RHC/FQHC updates, laboratory provisions, prescription-drug rebate administration, Medicare eligibility implementation, and an RFI on duplicate testing and interoperability. The comment period closes September 14, 2026.

Dataset and methodology

Collection

The census includes every record returned by the Regulations.gov comments search for the initiating document's internal object ID: 1,595 IDs across seven 250-record pages. It is exact for records publicly posted at snapshot time. Posted-date coverage was July 17 through August 3, 2026.

A systematic, seeded stratified sample of 560 IDs was drawn across the ordered census. Detail retrieval succeeded for 456 records before the public client quota was exhausted; 104 requests returned HTTP 429. The resulting sample includes every posting day and remains close to the census's date distribution. It should nonetheless be treated as a quota-constrained sample, not a randomized agency export.

Theme coding and estimates

The report combines structured keyword rules with qualitative reading of representative comments and near-duplicate clusters. Themes are multi-label: one comment may count toward payment adequacy, same-day E/M, and access. Estimated full-corpus counts multiply the sample rate by 1,595; 95% intervals use Wilson binomial intervals. These are directional estimates, not audited docket totals.

Identity and attachments

Submitter identity fields are agency-configurable and appear only in detail records. In the 456-record sample, 26 comments exposed an organization, 429 exposed a personal name field, and 1 exposed neither; 99 of the named-field records used 'Anonymous Anonymous' and were treated as withheld. Twenty-three sample records referenced at least one attachment (24 attachment objects). The attachment files themselves could not be extracted from the download host during the run, so attachment-only submissions are underrepresented in theme coding. Attachment titles were reviewed where available.

Theme prevalence

Table 1 reports sample incidence and directional estimates for the live 1,595-record census. Percentages sum to more than 100% because themes overlap.

Theme	Sample n	Sample %	Est. full corpus	Interpretation
Payment adequacy, conversion factors & practice expense	261	57.2%	913 (840–984)	Opposition to net payment reductions, outdated or volatile practice-expense inputs, and continued pressure on independent practices.
Access, rural burden & consolidation	217	47.6%	759 (687–832)	Claims that lower office-based payment will lengthen waits, shift care to hospital settings, increase travel, and accelerate consolidation.
Same-day E/M, modifier 25 & global procedures	179	39.3%	626 (556–699)	Strong objections to paying only 50% for additional same-day services; commenters predict split visits, added copays, and delayed diagnosis.
Specialty/service valuation disputes	51	11.2%	178 (137–230)	Code-specific requests involving arthroplasty, allergy immunotherapy, neuropsychology, maternity, speech-language services, and radiation oncology.
Chronic disease, health coaching & navigation	34	7.5%	119 (86–163)	Generally supportive requests for payment pathways that recognize coaching, navigation, shared visits, and caregiver-facing support.

Theme	Sample n	Sample %	Est. full corpus	Interpretation
Remote monitoring staffing, initiation & coding	27	5.9%	94 (65–135)	Concern that established-patient, initiating-visit, and employee-only staffing rules would narrow access and disrupt existing delivery models.
Laboratory, diagnostics & interoperability	25	5.5%	87 (60–127)	Comments on laboratory valuation, diagnostic technologies, result sharing, and duplicate testing.
Drug payment, pharmacies & 340B	8	1.8%	28 (14–55)	A small set addressing pharmacy access, drug affordability, rebate administration, or 340B reporting.
AI/CMAA payment and human oversight	7	1.5%	24 (12–50)	Objections to direct payment for autonomous clinical AI without equivalent recognition of human clinical staff and clear accountability.
Telehealth, RHCs & FQHCs	5	1.1%	17 (7–41)	A limited set focused on virtual access and rural or federally qualified clinic payment.
Wound care & skin substitutes	5	1.1%	17 (7–41)	Comments raising evidence, training, and payment concerns in wound care and skin-substitute policy.
Medicare eligibility/immigration	1	0.2%	3 (1–20)	Very limited sample attention to the rule's eligibility provisions.

Detailed thematic analysis

1. Payment adequacy and practice expense

Commenters repeatedly compare the proposed conversion factors with rising practice costs and the expiration of the temporary 2.5% CY 2026 statutory increase. Independent practices describe narrow margins, staffing pressure, cybersecurity and compliance expense, and reimbursement that has not kept pace with inflation. Several letters also criticize new practice-expense methods as volatile or insufficiently grounded in current specialty costs.

Policy requests observed. Common asks: restore or stabilize the conversion factor; use auditable, current cost data; phase in practice-expense changes; protect office-based care; and work with Congress on a permanent inflation-linked update.

2. Same-day E/M and global procedures

This is the most concentrated policy-specific opposition. Dermatology, orthopedics, urology, allergy, and other procedural specialties argue that a separately identifiable E/M service is clinically distinct even when furnished with a procedure. They predict the 50% rule will encourage separate appointments, increasing travel, caregiver burden, missed work, cost sharing, and diagnosis delays.

Policy requests observed. Common asks: withdraw the 50% multiple-procedure reduction; retain modifier 25 safeguards; target auditing at improper use rather than across-the-board payment; or exempt clearly documented, medically necessary services.

3. Access, rural burden, and consolidation

Access arguments function as the bridge between payment mechanics and beneficiary outcomes. Commenters expect fewer practices to accept Medicare, longer specialty waits, reduced rural availability, and faster acquisition of independent practices by hospitals or corporate groups. Several point out that shifting a service to a hospital outpatient setting may raise total Medicare and beneficiary spending even if the physician payment line falls.

Policy requests observed. Common asks: perform setting-of-care and rural-impact analyses; model beneficiary travel and wait-time effects; and avoid policies that reward consolidation.

4. Remote monitoring

Remote monitoring comments focus on proposals that services be limited to established patients, require an initiating visit, and be furnished by clinical staff employed by the practice rather than contractors. Supporters of current models argue that smaller practices often need specialized contracted staff and that tighter rules would reduce chronic-care capacity; others support stronger accountability and want the billing practice to retain responsibility for safety and supervision.

Policy requests observed. Common asks: permit qualified contracted clinical staff under documented supervision; clarify incident-to and care-team accountability; avoid bundling that obscures clinically distinct work; and preserve access for small or rural practices.

5. Specialty and service valuation

Code-level requests include total hip and knee arthroplasty, allergy immunotherapy (including 95165), psychological and neuropsychological testing, speech-language services, maternity payment, radiation oncology, and diagnostic technologies. The shared argument is that survey assumptions or time estimates understate clinical intensity, pre/post work, overhead, or patient complexity.

Policy requests observed. Common asks: accept specialty society evidence, reassess time and practice inputs, avoid abrupt reductions, and publish transparent crosswalks and impact files.

6. Coaching, navigation, and chronic disease

This cluster is more supportive. Nurse coaches and health coaches favor national valuation and payment pathways for coaching, navigation, shared medical appointments, and preventive chronic-care models. Some caregiver comments ask that CHI or PIN support extend directly to family caregivers when a beneficiary cannot participate fully.

Policy requests observed. Common asks: finalize coaching codes with recognized credentials; include nurses and qualified coaches; allow team-based models; and define measurable outcomes without excessive administrative burden.

7. AI, diagnostics, drugs, and other smaller themes

Comments on CMAA/autonomous clinical AI caution against paying algorithms while under-recognizing human staff and ask for accountability, validation, and transparency. Diagnostic stakeholders address test valuation and interoperability. Pharmacy and 340B comments focus on access and reporting burden. Telehealth and safety-net clinic comments are present but relatively sparse in this early snapshot; eligibility provisions draw very little sample attention.

Policy requests observed. Common asks: require human oversight and evidence for autonomous tools; improve result exchange to reduce duplicate testing; minimize duplicative reporting; and preserve virtual and safety-net access.

Campaign and duplication effects

The public record includes repeated or closely adapted language. In the sample, recognizable clusters included approximately 13 near-duplicate independent-practice payment letters, 7 physical-therapy payment letters, 10 dermatology same-day-care letters across two variants, 4 broad practice-sustainability letters, 4 arthroplasty letters, 4 nurse-coaching letters, 3 autonomous-AI letters, and additional two- or three-comment clusters. These counts identify sampled templates, not the full size of each campaign.

For decision-making, CMS should separate three units of analysis: record volume, unique or near-unique language, and distinct evidence or policy claims. A high-volume campaign is evidence of salience and mobilization, but it does not multiply the number of independent technical arguments.

Stakeholder profile

Public category (as posted)	Sample n	Share
Health Care Professional/Association - Physician	142	31.1%
Unspecified	111	24.3%
Physician - HC005	41	9.0%
Health Care Industry - PI015	22	4.8%
Individual	15	3.3%
Health Care Professional/Association - Nurse	15	3.3%
Health Care Professional/Association - Physical Therapist	9	2.0%
Academic - OT005	8	1.8%
Government - Federal	8	1.8%
Health Care Professional/Association - Speech-Language Therapist	7	1.5%
Congressional	7	1.5%
Health Care Professional/Association - Other Health Care Professional	6	1.3%

The sample is practitioner-heavy: physician and physician-coded categories alone account for a large share, while public category is unspecified for many submissions. Category labels are submitter- or agency-supplied and should not be treated as independently verified credentials.

Top 100 named submitters in the detail sample

Ranking rule: greatest observed number of detailed comment records, then alphabetical by public submitter name. Anonymous/withheld records are excluded. Because nearly all named submitters appear once, ranks 3–100 are alphabetical tie-breaks—not a measure of influence, representativeness, or full-docket frequency. This table cannot be generalized to all 1,595 records without the official bulk export.

Rank	Submitter	Type	Observed n	Comment ID(s)
1	David Doud	Individual	2	CMS-2026-2377-1298, CMS-2026-2377-1300
2	natwarlal ramani	Individual	2	CMS-2026-2377-0485, CMS-2026-2377-0486
3	About Women Ob/Gyn	Organization	1	CMS-2026-2377-1086
4	Ada West Dermatology	Organization	1	CMS-2026-2377-1326
5	Adam Wright MD	Individual	1	CMS-2026-2377-0197
6	Addie Taylor	Individual	1	CMS-2026-2377-1373
7	Adrienne Jordan	Individual	1	CMS-2026-2377-0522
8	Alaa Kassir	Individual	1	CMS-2026-2377-0149
9	Alexi Johnson	Individual	1	CMS-2026-2377-0831
10	Alison Auster	Individual	1	CMS-2026-2377-1665
11	Alison Black	Individual	1	CMS-2026-2377-0898
12	Allie Nitsch	Individual	1	CMS-2026-2377-0526
13	Allie Pardue	Individual	1	CMS-2026-2377-0708
14	Alyse Worley	Individual	1	CMS-2026-2377-1639
15	Amanda Bailey	Individual	1	CMS-2026-2377-1097
16	Amar Patel	Individual	1	CMS-2026-2377-0810
17	Amir Aboutalebi	Individual	1	CMS-2026-2377-0937
18	Amy Brackeen	Individual	1	CMS-2026-2377-0450
19	Amy Holton	Individual	1	CMS-2026-2377-1184
20	Amy McGaha	Individual	1	CMS-2026-2377-1401
21	Amy Schulz	Individual	1	CMS-2026-2377-0134
22	Anacortes Dermatology Inc	Organization	1	CMS-2026-2377-1224
23	Anand Gupta	Individual	1	CMS-2026-2377-0275
24	Andrea Lausch	Individual	1	CMS-2026-2377-1081
25	Andrew Wallner	Individual	1	CMS-2026-2377-1430
26	Angela Ellerman	Individual	1	CMS-2026-2377-0156
27	Angela Wipf	Individual	1	CMS-2026-2377-0551
28	Angelina Bradley	Individual	1	CMS-2026-2377-1474
29	Ann Gerner	Individual	1	CMS-2026-2377-1105
30	Ann Riehl	Individual	1	CMS-2026-2377-0332
31	Anna Desmarais	Individual	1	CMS-2026-2377-0113
32	Anne Banfield	Individual	1	CMS-2026-2377-1119
33	Annette Angiuli	Individual	1	CMS-2026-2377-0298
34	Annie Reybrock	Individual	1	CMS-2026-2377-1386
35	Ariel Ouziel	Individual	1	CMS-2026-2377-0716
36	Arjun Dupati	Individual	1	CMS-2026-2377-0234
37	Ashley Myers	Individual	1	CMS-2026-2377-0369
38	Ashutosh Barve	Individual	1	CMS-2026-2377-0368
39	Atlanta Obstetrics and Gynecology Specialists	Organization	1	CMS-2026-2377-1382
40	Audrey Weissman	Individual	1	CMS-2026-2377-1309
41	Austin Butler	Individual	1	CMS-2026-2377-0946
42	Awa Sanneh	Individual	1	CMS-2026-2377-1039
43	B G	Individual	1	CMS-2026-2377-0431
44	Basil Kahwash	Individual	1	CMS-2026-2377-1331
45	basir tareen	Individual	1	CMS-2026-2377-0593
46	Benjamin Perry	Individual	1	CMS-2026-2377-0094

Rank	Submitter	Type	Observed n	Comment ID(s)
47	Berry Jiang	Individual	1	CMS-2026-2377-1556
48	Bill Cosker	Individual	1	CMS-2026-2377-0607
49	Blaize O'Brien	Individual	1	CMS-2026-2377-1654
50	blake schultz	Individual	1	CMS-2026-2377-1219
51	Blue Valley Dermatology	Organization	1	CMS-2026-2377-0099
52	Bonnie Pewterbaugh	Individual	1	CMS-2026-2377-0749
53	Brandon Hirsch	Individual	1	CMS-2026-2377-0199
54	Brenda Kelley	Individual	1	CMS-2026-2377-0752
55	Brenda Proctor	Individual	1	CMS-2026-2377-1367
56	Brett Kalmowitz	Individual	1	CMS-2026-2377-0355
57	Brian Kleiber	Individual	1	CMS-2026-2377-1034
58	Brian Sanders	Individual	1	CMS-2026-2377-1560
59	Bridget Kelleher	Individual	1	CMS-2026-2377-0774
60	Brielle Plost	Individual	1	CMS-2026-2377-0181
61	Brittany Parrill	Individual	1	CMS-2026-2377-0616
62	Burlington ENT PC	Organization	1	CMS-2026-2377-1547
63	Camilo Fernandez-Salvador	Individual	1	CMS-2026-2377-1244
64	Candis Davis	Individual	1	CMS-2026-2377-0879
65	Cara Milam	Individual	1	CMS-2026-2377-0349
66	Carey Amsden	Individual	1	CMS-2026-2377-1580
67	Carlton Phillips	Individual	1	CMS-2026-2377-1142
68	Carol Van Egeren	Individual	1	CMS-2026-2377-0525
69	Carson Teague	Individual	1	CMS-2026-2377-1425
70	Casey Barry	Individual	1	CMS-2026-2377-1230
71	Casey Timmerman	Individual	1	CMS-2026-2377-0159
72	Catherine Draper	Individual	1	CMS-2026-2377-1656
73	CHAN NGUYEN	Individual	1	CMS-2026-2377-0167
74	Channa Ovits	Individual	1	CMS-2026-2377-0488
75	Charles Mitchell	Individual	1	CMS-2026-2377-1468
76	Charles Patterson	Individual	1	CMS-2026-2377-1130
77	Charles Phipps	Individual	1	CMS-2026-2377-0822
78	Chris Livingston MD FACS	Individual	1	CMS-2026-2377-0236
79	Christina Brennan	Individual	1	CMS-2026-2377-1208
80	Christine Giustra	Individual	1	CMS-2026-2377-1123
81	Christine Patterson	Individual	1	CMS-2026-2377-1613
82	Christine Yoder	Individual	1	CMS-2026-2377-1449
83	Christopher Ha	Individual	1	CMS-2026-2377-0746
84	Cindy Wassef	Individual	1	CMS-2026-2377-0267
85	Code Pause, Inc.	Organization	1	CMS-2026-2377-0131
86	Conchita Bassetti	Individual	1	CMS-2026-2377-0118
87	Connie Lichtenwald	Individual	1	CMS-2026-2377-1127
88	Consuelo David	Individual	1	CMS-2026-2377-0834
89	Crystal Loucel	Individual	1	CMS-2026-2377-1618
90	Cynthia Harrington	Individual	1	CMS-2026-2377-0126
91	D'nese Sokolowski	Individual	1	CMS-2026-2377-1113
92	Dana Al-Khaled	Individual	1	CMS-2026-2377-0505
93	Daniel Fox	Individual	1	CMS-2026-2377-1434
94	Daniel Lo	Individual	1	CMS-2026-2377-1022
95	Daniel Nadelman	Individual	1	CMS-2026-2377-0807
96	Daniel Okamoto	Individual	1	CMS-2026-2377-1232
97	Daniel Rothbaum	Individual	1	CMS-2026-2377-1157
98	Daniel Verret	Individual	1	CMS-2026-2377-1357
99	David Bianchi	Individual	1	CMS-2026-2377-0723
100	David Gustafson	Individual	1	CMS-2026-2377-1633

Limitations and open questions

- The docket remains open through September 14, 2026; this is an interim snapshot, not the final administrative record.
- Theme estimates come from 456 detail records, not the entire census. Quota failures may introduce residual selection bias despite broad date coverage.
- Attachment files were not extracted; attachment-only comments and detailed exhibits are undercoded.
- Identity and category fields are public but agency-configurable, self-reported, and sometimes withheld. Names were not independently verified or entity-resolved.
- Near-duplicate detection was conservative and sample-based; exact campaign sizes require full-text access to every record.
- Multi-label keyword rules can produce false positives and false negatives. Estimated counts are directional ranges, not legal or statistical findings by CMS.

Recommended next step for a final report

After the comment period closes—and after allowing time for agency posting and moderation—request the official Regulations.gov bulk export for document CMS-2026-2377-0002. Re-run entity normalization, attachment extraction/OCR, exact near-duplicate clustering, and full-corpus theme coding. Replace the sampled top-100 appendix with an audited census ranking and publish both record-volume and unique-language views.

Sources

[Regulations.gov document and live comments](#) — Primary public docket interface and live record count.

[Regulations.gov API documentation](#) — API field definitions, public-data limitations, detail and search behavior.

[CMS CY 2027 PFS proposed rule fact sheet](#) — Official overview of major proposals and conversion factors.

[CMS-1848-P regulation page](#) — Official rule materials, downloads, dates, and supporting files.

Rerun inputs

workflow: firecrawl-deep-research (official API fallback; Firecrawl credential unavailable)

topic: Comments on CMS-2026-2377-0002

depth: 30-minute thorough/exhaustive hybrid

snapshot: 2026-08-03 20:00 EDT

census: 1595 public comment records

detail sample: 456 records (seed 20260803)